



BUSINESS ASSETS

VYTALIX — Company Profile

Vytalix company-build documentation · reference COMPANY_PROFILE

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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Contents

Executive view	1
1. Who we are	2
2. What we do — four pillars	3
3. How we work	4
4. MaternaLink (in development)	5
5. Principles we will not trade	6
6. Contact	7
Priorities / Risks / Next actions	8

Technology for Life · Two-page profile · Version 1.0 · 8 September 2026 · Status: PROPOSED (external-ready once the bracketed placeholders are filled and the name is cleared)

Governed by `/FACTS_BASE.md` . Tone per `/docs/03_BRAND_SYSTEM.md` . Nothing below is a claim of achievement; every forward statement is labelled.

Executive view

1. Vytalix is a pre-seed, founder-led UK health-technology group with four connected pillars — Advisory, Consulting, Health Solutions and E-Learning — designed so that advice informs delivery, delivery informs product, and product informs learning.
2. What we can do for a client today is founder-led advisory and fixed-scope consulting on digital health strategy, NHS readiness, UK–Africa market entry, funding readiness and digital maternity; E-Learning follows once the first courses are built (ESTIMATE: month 9).
3. Our first product, MaternaLink (in development), is a non-diagnostic maternal care-coordination, communication, education and monitoring-support platform for NHS and low-connectivity settings; it is not validated, not approved and not yet deployed anywhere.
4. We are honest about stage: no clients, partners, approvals or team beyond the founder are claimed; credibility comes from rigour, safety-first scoping and published thinking (see `/docs/14_MARKETING_AND_CONTENT_ENGINE.md`).
5. Use this profile as the "About Vytalix" appendix in proposals, decks and the website; update the placeholders after incorporation and name clearance (`/docs/02_GROUP_STRUCTURE.md` §7).

1. Who we are

Vytalix is a UK health-technology group that advises, builds, delivers and teaches, so that technology earns its place in care.

We are founder-led and pre-seed. We are building the company in the open and we say exactly what we can do today. Our first product focus is maternal health, where outcomes are unequal and coordination fails too often — in the UK, where public inquiries keep finding the same failures to listen, escalate and hand over, and in Nigeria, which carries the largest absolute maternal-death burden in the world (sources to be cited to the current MBRRACE-UK and WHO reports before external use; see `/docs/04_MATERNALINK_PRODUCT_STRATEGY.md` §2).

Vision (PROPOSED): Technology for Life. A world in which every person — beginning with every mother — is connected to care that knows them, hears them and acts in time, wherever they live.

Mission (25 words, PROPOSED): Vytalix is a UK health-technology group that helps health organisations, founders and governments make technology work in real care — beginning with maternal care coordination.

Stage, plainly (KNOWN): pre-formation; legal entity, name clearance and registrations in progress; no revenue; no product in production. Everything we publish is labelled KNOWN, ESTIMATE, PROPOSED or TO VALIDATE.

2. What we do — four pillars

Pillar	What it is	Who it is for	What is true today
Vytalix Advisory	Independent, senior advice on health-technology strategy, market entry (UK ↔ Africa), regulation and funding readiness: advisory days, monthly retainers, fractional Chief Digital/Strategy Officer	Health-tech founders (pre-seed to Series A), SME providers, NGOs, ministries via donors, investors and accelerators	Founder-led; independent (no vendor commissions); available now
Vytalix Consulting	Fixed-scope, fixed-price delivery: discovery sprints, business cases, digital maternity readiness, DTAC and evidence readiness, market-entry playbooks, health-inequalities reviews, programme evaluation	NHS trusts and ICBs (via frameworks or sub-contract), private providers, health-tech vendors, NGOs and foundations	Ten packaged offers with defined deliverables (/assets/SERVICE_CATALOGUE.md); documented methods
Vytalix Health Solutions	Products designed for real care settings, starting with MaternaLink (in development)	NHS trusts and LMNS, private maternity, employers; African state programmes, NGOs, insurers	An early prototype exists; a safety-first, non-diagnostic v1 scope; a roadmap with explicit validation gates
Vytalix E-Learning	Practical learning on digital health, care coordination and safe AI for health teams and founders: self-paced courses, subscription library, corporate and university licences	Midwives, nurses, health-tech professionals, students (UK and Africa); employers; universities	Curriculum themes and formats defined; first two courses planned for month 9 (ESTIMATE); no accreditation held or claimed

The pillars are one company, not four. A readiness review, a business case, a pilot and a training programme are designed to fit together.

3. How we work

- **Fixed scope, fixed price, written change control.** Every consulting engagement has a defined deliverable, an acceptance criterion and a start and end date.
- **Evidence before claims.** We cite sources or we do not use the number. We tell you if your idea should not be built.
- **Safety before speed.** For anything that touches patients we start from the intended-use statement, the clinical-safety approach (DCB0129/DCB0160) and the data-protection impact assessment, not from features.
- **Build with, not for.** Discovery interviews with the people who will use the thing come before design. We only say "built with clinicians" once clinicians have actually co-designed it.
- **Both markets, one standard.** UK home market; active interest in maternal health in Nigeria and one further African market. The same safety, data and honesty rules apply in both.
- **Stage-honest.** We are pre-seed and founder-led. Associates are engaged only on signed work and named only with their consent.

Values in one line each (PROPOSED): Truth before traction · Safety before speed · Build with, not for · Both markets, one standard · Evidence is the product.

4. MaternaLink (in development)

What it is intended to be: a care-coordination and communication platform for maternity services, designed to help women stay connected to their care team and help care teams hand over safely — built for both NHS and low-connectivity settings (web app plus SMS and voice channels).

v1 scope (PROPOSED): enrolment and consent; personalised contact schedules and reminders; clinically signed-off education in the woman's language; secure two-way messaging with a named human owner and acknowledgement time; a self-recorded diary that is displayed, not interpreted; a care-team console; a structured handover summary; an escalation log; missed-contact lists; a programme dashboard.

What it is not: it does not diagnose, predict, triage or score. It is designed to support — not replace — clinical judgement. The multimodal AI risk-scoring concept from 2025 is held as a research track (v3+) that advances only with data partnerships, ethics approval, clinical validation and a UK MDR / MHRA software-as-a-medical-device pathway.

Status (KNOWN): early prototype of unknown maturity; intellectual-property ownership being documented; no data, model, validation, clinical partner, ethics approval or deployment. Four gates must close before anything is sold: IP documented, prototype audited, Clinical Safety Officer appointed and hazard log opened, DPIA completed (/docs/04_MATERNALINK_PRODUCT_STRATEGY.md , Executive view).

What we are looking for: one UK maternity service willing to act as an unpaid design partner for a service evaluation; one Nigerian implementing partner with an existing cohort; clinical advisers (obstetrics, midwifery, clinical safety). Partnership conversations are described in /assets/PARTNERSHIP_DECK.md .

5. Principles we will not trade

1. We never describe MaternaLink as proven, NHS-approved, clinically validated, CE/UKCA-marked, MHRA-registered or as reducing maternal mortality.
2. We never name a client, partner, adviser or team member without written consent and a real relationship.
3. Advisory and consulting work is designed to process no patient data.
4. Every number in our material is sourced or labelled ESTIMATE.
5. Conflicts of interest are declared in writing (the founder's separate venture, Kemek Enterprise Ltd, is a declared interest).

6. Contact

- Founder: [Founder name — confirm display name] · [Role title]
- Email: [hello@domain — TO VALIDATE after domain purchase]
- LinkedIn: [Founder profile URL] · [Vytalix company page URL — created after name clearance]
- Website: [URL — live after incorporation; source in /website]
- Registered office: [Registered office address — add after incorporation]
- Legal entity: Vytalix is a trading name of [Legal entity name — TO VALIDATE; recommended "Vytalix Group Ltd"], registered in England & Wales, company no. [●]. VAT status: [TO VALIDATE].
- Phone: [Not published until a business line exists]

Priorities / Risks / Next actions

Priorities

1. Clear the name and incorporate (/docs/02_GROUP_STRUCTURE.md §7), then fill every placeholder above and re-label this profile from PROPOSED to ADOPTED.
2. Use the profile unchanged as the appendix to every proposal and deck so that the stage disclosure travels with every claim.
3. Recruit the first clinical adviser so that "built with clinicians" can become true, with consent to be named.

Risks

- A rename after clearance would require a find-and-replace across all assets; the brand system is designed for this, so no printed assets are commissioned before clearance.
- Any drift toward "proven platform" language in derivative copy; mitigation: FACTS_BASE checklist before publishing.
- Contact placeholders left in an external document; mitigation: a pre-send check in /docs/13_SALES_ENGINE.md §10.

Next actions

- Founder, by Day 10: name clearance result; incorporation; domain; email.
- Founder, by Day 14: placeholders filled; profile exported to PDF on the letterhead specified in /docs/03_BRAND_SYSTEM.md §14.
- Founder, by Day 60: first adviser named (with consent) in §4.